

Public Health Emergency Operations Centre

"COUSP-DRC"

Ebola Virus Disease Situation Report/ 17th

Epidemic

SitRep MVE No. 002/2026

Country/Province	DRC/Ituri
Affected Health Zone	Mongbwalu, Bunia and Rwampara
Reporting date	May 17, 2026
Publication date	May 18, 2026

340	336	13	246	ND	ND	ND	0
Alerts	Alerts investigated	Case Confirmed	Suspicious Cases	Deaths confirmed	Bed occupancy rates among confirmed patients	Bed occupancy rates among suspected cases	Healed

I. KEY POINTS

- ☒ Declaration of the 17th Ebola outbreak in the health zones of Bunia, Rwampara and Mongbwalu in the province of Ituri by His Excellency the Minister of Health, Hygiene and Social Welfare (May 15, 2026).
- ☒ The response plan for the 17th Ebola virus disease outbreak is currently being developed;
- ☒ Investigation and sampling of a suspected case in Kinshasa coming from Butembo / North Kivu;
- ☒ 3 ZS/36 are affected including Bunia, Rwampara and Mongbwalu.
- ☒ 13 cases tested positive out of 20 samples taken in the Rwampara health zone for Ebola outside of Zaire. (Sudan Ebola virus)



Context and justification

The COUSP/DRC received an alert reporting a group of deaths of unknown etiology in the Mongbwalu Health Zone (90 km from Bunia), Djugu territory in the Ituri province, which is located in the eastern part of the Democratic Republic of Congo.

A preliminary health alert dated May 5, 2026, originated from healthcare personnel in the Mongbwalu health zone and was officially reported on May 8 following reports of unexplained cluster deaths (50 deaths via social media). Between April 1 and May 13, 2026, 261–280 suspected and probable cases, including 100 deaths (55 in-hospital and 45 community deaths in the Mongbwalu health zone), were reported, including 4 deaths, all of whom were healthcare professionals. All these cases originated from the Mongbwalu, Rwampara, and Bunia health zones. The outbreak is located in the Shun (or Sun 1) neighborhood of Mongbwalu (Alimasi 1 and 2 avenues).

The event reportedly occurred amid strong suspicions of a community outbreak, initially marked by rapid and unexplained deaths, some of which were linked to risky body handling practices and funeral rituals and prayer groups.

After reviewing in-hospital deaths, some common clinical signs such as fever, headache, vomiting, intense physical asthenia in the majority of cases and dyspnea, hematemesis, epistaxis, blackish vomit in other cases were noted.

The investigations carried out by the EIR resulted in 28 samples being taken from suspected cases. Initially, preliminary tests carried out at INRB/Bunia were all negative for Ebola zaire, dengue, Rotavirus, Cholera, Malaria, Yersinia pestis, MPOX and COVID-19 before 13 of these samples were sent to INRB/Kinshasa for further analysis.

As of May 14, 2026, **eight cases have tested positive for Ebola virus disease of the Bundibugyo/Uganda strain**, identified after sequencing. This confirms the **17th Ebola virus disease outbreak in the DRC**.

II. EPIDEMIOLOGICAL SITUATION UPDATE

Between April 1st and May 14th, 2026, 256 patient records were compiled at the Mongbwalu General Referral Hospital, including 141 cases in April and 115 cases in May. A total of 57 deaths were recorded, 34 in April and 23 in May.

Following a meeting with the five health officers from the CECA 20, Notre Dame, Abelkoko, Ituri, and Mongbwalu health areas, 45 community deaths were reported, including 26 in CECA 20 and 19 in Mongbwalu. Among these deaths, 15 occurred within the same family in the Mongbwalu health area, including 5 in Bunia, after a family gathering, over a period of two weeks.

The deceased individuals presented with the same clinical picture of headaches, fever, and vomiting. According to the community, these deaths are linked to a story called "Tumu," related to fetishes that were allegedly burned by a pastor, resulting, according to local beliefs, in his death and that of his wife.

The situation is currently being investigated by the team.

Spatial distribution of affected Health Zones

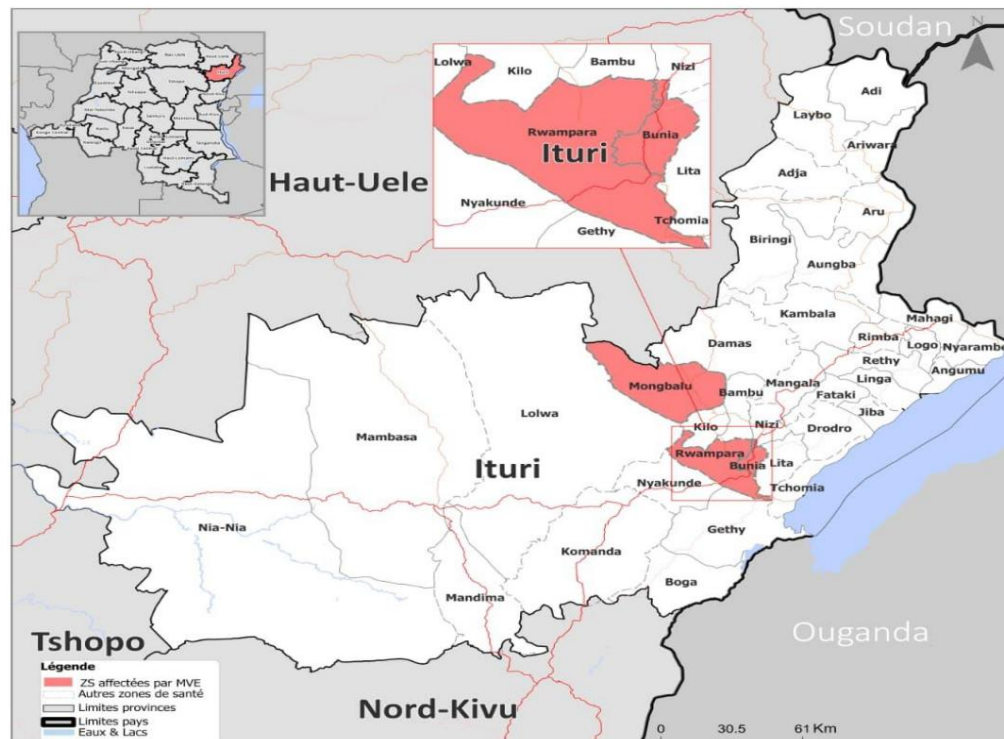


Figure 1: Mapping of affected health zones in terms of suspected (+confirmed) cases

Cumulative cases as of May 15, 2026

Table 1. Distribution of alerts received, validated and investigated by Health Zone

Indicators	ZS Mongbalu	ZS Rwampara	ZS Bunia 256	Total
Alerts received		72	12	340
Cases investigated	256	72	8	336
Suspected cases sampled	7	25	0	32
Case Confirmed	0	13	0	13
Case Probables	57	27	3	87
Case Suspects in isolation	3	5	4	12
Suspected cases in progress, of sample collection	0	5	6	11

ACTIVITIES CARRIED OUT

III. COORDINATION

- ☒ Holding of CGUS meetings with the Mayor of the Mongbalu area;
- ☒ Holding of the bipartite meeting: WHO and Provincial Government;
- ☒ Holding of 2 COUSP Provincial meetings;
- ☒ The Incident Action Plan (IAP) is currently being developed;
- ☒ National COUSP meeting held regarding the emergency situation

IV. EPIDEMIOLOGICAL SURVEILLANCE

- ☒ Official notification of the alert on May 8, 2026 following rumors of group deaths.
- ☒ Update on the epidemiological situation and linear lists of suspected and probable cases
- ☒ Sample collection, analysis and shipment of samples to the INRB

V. LABORATORY

- ☒ Thirteen initial samples were taken, two of which were analyzed by rapid diagnostic test (RDT) with negative results (Dengue, MPOX, Malaria, *Yersinia pestis*, COVID-19, Rotavirus and Cholera),

VII. INFECTION PREVENTION AND CONTROL (PCI/WASH)

- ☒ PCI evaluation, CS Abelkoko (score 7%), HGR Mongbwalu (score 34%).

IX. RISK COMMUNICATION AND COMMUNITY ENGAGEMENT

- ☒ Initial awareness-raising of community and religious leaders to counter rumors of "mystical illness".
- ☒ Drafting of the message from the Military Governor regarding the prevention of the Ebola virus disease which has claimed lives in this province.
- ☒ Development of educational messages, tailored to specific target audiences, on the prevention of this PCI disease:
Continued assessments of the conditions for implementing Prevention and Control
Infections in healthcare facilities, Logistics: Preparatory
- ☒ meeting to assess logistical needs for all pillars.

X. SECURITY

- ☒ Daily security monitoring.

XII. CHALLENGES AND GAPS

- ☒ Lack of compliant isolation, development of the PAI and designation of SGI;
- ☒ Lack of PCI input, sampling kits;
- ☒ CREC needs to manage community rumors;
- ☒ Personnel not trained in IPC, sample collection, transport and storage, surveillance, management of disease with epidemic potential;
- ☒ Lack of triage at HGR Mongbwalu and CS;
- ☒ Complete EDS implementation;
- ☒ Urgent need for standardized CTE in the 3 ZS during epidemics.

XIII. PERSPECTIVES

- ☑ *Organize and standardize Dignified and Safe Burials (DSBs) to neutralize the major source of contamination represented by bodies.*
- ☑ *Infection Prevention and Control (IPC) kits, sample collection*
- ☑ *and transport, training of service providers on epidemiological surveillance, IPC,*
- ☑ *CREC, strengthening community-based surveillance,*
- ☑ *increasing the number of epidemiological surveillance tools, communication, ...*
- ☑ *Identification, rehabilitation and/or redevelopment of care structures;*
- ☑ *Strengthening of epidemiological surveillance at points of entry,*
- ☑ *Initiate immediate cross-border coordination with Uganda and South Sudan to monitor population movements at the borders.*
- ☑ *Activation of the Health Emergency Operations Centre*



Press briefing by the Minister of Health



National Coordination Meeting, COUSP-DRC



Provincial Coordination Meeting, DPS Ituri

FOR ALL
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PLEASE CONTACT

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